European Journal of Trauma & Dissociation 10 (2026) 100685

Contents lists available at ScienceDirect

European Journal of Trauma & Dissociation

journal homepage: www.elsevier.com/locate/ejtd

Research Paper

Coping School: A proof-of-concept feasibility study of a primary care psychoeducational group promoting self-care after adverse events

Amanda Karlsson [a], Charlotte Bäccman [b,c], Maria Unenge Hallerbäck [d], Siri Jakobsson Støre [b,c,*]

[a] Capio Primary Care, Region Värmland, Sweden
[b] Department of Social and Psychological Studies, Karlstad University, Karlstad, Sweden
[c] The Service Research Center, Karlstad University, Karlstad, Sweden
[d] School of Medical Sciences, Örebro University, Örebro Sweden

ARTICLE INFO

Keywords:
Coping
Open group intervention
Primary care
Posttraumatic stress
Proof-of-concept
Psychoeducation
Trauma

ABSTRACT

Introduction: PTSD is defined by the presence of a qualifying traumatic event (Criterion A), yet considerable debate remains regarding how this criterion should be operationalized, as individuals exposed to non–Criterion A stressors may develop clinically significant PTSD-like symptoms. This diagnostic complexity poses challenges for identification and treatment of trauma-related distress in primary care settings.
Objective: The aim of the study was to investigate whether *Coping School*, a psychoeducational group for patients and their relatives, is feasible and associated with symptom reduction, how participants experience the intervention, and therapists' perspectives on recruitment difficulties.
Method: Eleven assessment interviews with potential participants were conducted, of which only four were deemed eligible. The study used a within-group design with measurements conducted pre, mid, and post intervention, and three-month follow-up.
Results: At follow-up, changes in symptom ratings relative to baseline varied across participants. Participants perceived the intervention as valuable for increasing self-understanding, but that an overly condensed intervention led to the loss of important therapeutic elements. Therapists were also interviewed (n = 11) regarding their views on the intervention and factors that may have contributed to recruitment difficulties. The thematic analysis resulted in three themes: A limited eligible patient population, Hesitation toward group treatment, and Organizational barriers to collaboration and development.
Conclusions: These preliminary findings suggest that Coping School may be feasible and acceptable for some participants, but recruitment challenges – including how to engage and motivate other therapists to refer patients – highlight the need for further development and larger studies to evaluate effectiveness and generalizability.

1. Introduction

Approximately 10% of individuals exposed to trauma develop post-traumatic stress disorder (PTSD) (National Board of Health & Welfare, 2021). Research has shown that individuals suffering from PTSD tend to seek more medical care than those without PTSD (Kartha et al., 2008). Studies have shown that the risk of developing PTSD increases in the absence of adequate social support following a traumatic event (Vogt et al., 2007), and that social support influences the effectiveness of cognitive behavioral therapy (CBT) for PTSD (Fredette et al., 2016). It may therefore be valuable to enhance relatives' capacity to provide effective support once PTSD symptoms have been identified by health care services.

Numerous studies have demonstrated robust support for CBT-based therapies. In their systematic review and meta-analysis, Lewis et al. (2020) found that individual trauma-focused CBT (across several manualized treatments) and eye movement desensitization and reprocessing (EMDR) have the strongest evidence for effectiveness, while non–trauma-focused CBT and trauma-focused group CBT both show some effect. Schnyder et al. (2015) highlight psychoeducation, emotional regulation, and coping skills as common components of the most well-established and evidence-based trauma treatments. In addition to

* Corresponding author at: Department of Social and Psychological Studies, Karlstad University, SE-651 88 Karlstad, Sweden.
E-mail address: siri.store@kau.se (S.J. Støre).

https://doi.org/10.1016/j.ejtd.2026.100685
Received 14 February 2026; Received in revised form 10 April 2026; Accepted 18 April 2026
Available online 19 April 2026
2468-7499/© 2026 The Authors. Published by Elsevier Masson SAS. This is an open access article under the CC BY license (http://creativecommons.org/licenses/by/4.0/).

the aforementioned approaches, the transdiagnostic treatment manual *Unified Protocol* has also been shown to be effective for patients with trauma-related psychopathology, with or without meeting diagnostic criteria for PTSD (Hood et al., 2021; O'Donnell et al., 2021). The National Center for PTSD and the National Child Traumatic Stress Network have jointly developed *Skills for Psychological Recovery* (SPR), a brief manualized intervention focusing on building six core skills to promote recovery and resilience (National Center for PTSD & National Child Traumatic Stress Network, 2020). This intervention is designed for delivery in settings such as natural disasters, war, and terrorism, where conditions for standard psychotherapy are often lacking. Interpersonal psychotherapy (IPT), which focuses on improving interpersonal functioning, has also been adapted into a protocol for PTSD treatment without exposure and has demonstrated effectiveness comparable to prolonged exposure in available studies (Bleiberg & Markowitz, 2019). According to Swedish national guidelines, PTSD should primarily be treated with exposure-based CBT (National Board of Health & Welfare, 2021). Treatment attempts for mild to moderate conditions can generally be conducted within primary care.

Are et al. (2021) discuss evidence-based interventions for PTSD in primary care and highlight the potential of effective and cost-efficient interventions that begin at a "low dose" and gradually increase in intensity and scope for those who do not benefit sufficiently – so-called stepped care models of treatment. Stepped care is a model used in primary care mental health services to match the level of intervention to the individual's needs. The idea is to provide fewer intensive interventions to many patients initially and to gradually increase the intensity or offer alternative treatments to those who do not benefit from lower-intensity care (Center for Mental Health Competence, 2023). A common first step is guided self-help, group treatment, or psychoeducational lectures.

Silberleitner et al. (2021) suggest that identifying patient populations who currently do not receive optimal care at a given primary care clinic may be a useful starting point, as conditions and needs can vary substantially between clinics. They further recommend that group interventions ideally have rolling admission to optimize accessibility. Bowen et al. (2009) emphasize the importance of feasibility studies for making informed and well-founded decisions regarding which interventions are relevant and sustainable for implementation in public health care settings. The Swedish Psychological Association (2020) describes evidence-based psychological practice (EBPP) as a dynamic interplay between psychological research and professional practice, rather than a linear process in which research produces ready-made methods that are simply implemented. Within EBPP, research-informed practice is characterized by a scientific approach, whereby psychologists not only apply existing knowledge but also actively contribute to the development of new knowledge, for example through systematic and continuous evaluation of their own professional practice.

Unlike many other psychiatric diagnoses, PTSD and other trauma- and stress-related disorders require a link between a specific event and the subsequent onset of symptoms (Michel et al., 2018). The two most influential diagnostic systems -the *Diagnostic and Statistical Manual of Mental Disorders* (DSM: American Psychiatric Association, 2022) and the *International Classification of Diseases* (ICD: World Health Organization, 2025) – use different criteria for PTSD. As a result, some individuals may meet diagnostic criteria according to one system but not the other (Hyland et al., 2016; Stein et al., 2014; Hafstad et al., 2017). The formulation of Criterion A (the nature of the stressor) in the DSM has been debated since its introduction in DSM-III in 1980 (Marx et al., 2023). The criterion has been revised in subsequent editions of the manual. For example, earlier versions included the subjective experience of the event as extremely stressful or frightening, but this requirement has since been removed. Nevertheless, even in the updated DSM-5-TR, the individual's perception of the situation is still considered relevant in certain contexts, such as medical procedures or exposure to bullying (American Psychiatric Association, 2022). Marx et al. (2023) describe the debate about Criterion A and identify four dominant positions: expanding the criterion, narrowing the criterion, removing Criterion A entirely, or retaining it in its current form. Several studies and systematic reviews indicate that events not meeting Criterion A – such as divorce, financial hardship, racism, or discrimination – may nevertheless lead to PTSD-like symptoms. Proponents of a narrower criterion argue that qualifying through indirect exposure (e.g., learning about a loved one's life-threatening accident) does not align well with the theoretical rationale or treatment approaches used for PTSD. They emphasize the importance of distinguishing between normal reactions to adverse events – even those involving intense distress – and reactions that can be considered pathological and thus require treatment.

Experiencing a severe adverse event at some point in life is relatively common. For example, Frans et al. (2005) found that 80.8% of a randomly selected sample of Swedish men and women reported having experienced at least one traumatic event. However, individuals' responses to traumatic events vary widely. Southwick et al. (2023) describe resilience as a process that develops over time and may fluctuate across the lifespan. Adapting to stress does not preclude experiencing strong emotional reactions following an adverse event. Common themes among individuals who successfully adapt after trauma include prioritizing health and well-being, accepting social support, and confronting fears. Michel et al. (2018) identify lack of social support as the most important risk factor for developing long-term stress reactions after a traumatic event. The American Psychiatric Association likewise emphasizes social support as a significant factor both before and after traumatic exposure (DSM-5-TR; American Psychiatric American Psychiatric Association, 2022). It is important to recognize that coping may vary depending on the surrounding environment. A severe adverse event occurs within a context that includes close relatives and society at large. Ungar (2013) reviews research findings suggesting that the environment surrounding the affected individual influences coping capacity following trauma. Although individual characteristics play a role, Ungar argues that caregiving and upbringing have a greater impact than individual biology. While public health care systems cannot provide a positive and supportive social network, they can intervene to improve the conditions under which existing networks function. Such efforts are likely to benefit both the affected individual's and the relative's mental health.

The overall aim of the present study was to examine the feasibility of the Coping School, a psychoeducational group intervention for patients and their relatives in primary care, and to explore whether participation was associated with changes in trauma-related symptoms. The study further explores clinicians' views of the intervention and their perceptions of barriers contributing to recruitment difficulties. The research questions were: (1) Is the Coping School a feasible intervention for patients experiencing trauma-related symptoms and their relatives in primary care? (2) Is participation in the Coping School associated with changes in trauma-related symptoms over time? (3) How do participants experience the content and format of the Coping School? (4) How do clinicians perceive the Coping School and what factors do they identify as influencing recruitment challenges?

2. Material and methods

2.1. Study design

The study employed a mixed-methods design. Quantitative data were collected using self-report measures administered pre, mid, and post intervention, as well as three-month follow-up. Qualitative data were collected through semi-structured interviews with participants and clinicians. Quantitative and qualitative data were analyzed separately and then integrated to provide a comprehensive understanding of feasibility, outcomes, and implementation processes. The study was approved by the Swedish Ethical Review Authority and registered

retrospectively with Open Science Framework.

2.2. Setting

This naturalistic study was conducted in a primary care unit offering psychological services to adult participants between January 2024 and December 2025, in a medium-sized Swedish city with a municipality of approximately 90,000 inhabitants. Recruitment took place through clinicians at participating primary care units.

2.3. Participants

Patients were eligible to participate if they experienced psychological symptoms following one or more adverse events and were assessed by a psychologist in primary care as suitable for participation in a psychoeducational group. A formal diagnosis of PTSD was not required. Relatives of participating patients were also eligible to participate in the intervention. Exclusion criteria included current psychosis, severe substance use disorder, acute suicidality, or cognitive impairment that would impede participation in a group intervention. A total of eleven assessment interviews were conducted with potential participants. Of these, four patients met the inclusion criteria and consented to participate in the intervention. Three of these participants completed the intervention and were included in the follow-up assessment.

Clinicians working in primary care who had contact with patients potentially eligible for the intervention were invited to participate in interviews regarding their perspectives on the Coping School and the recruitment process. Eleven clinicians participated in these interviews, representing different professional backgrounds within primary care.

2.4. Intervention: The Coping School

The Coping School is a brief psychoeducational group designed for patients experiencing symptoms following adverse events, and their relatives. The intervention focuses on increasing understanding of trauma-related symptoms, enhancing coping strategies, and strengthening resilience and self-care. The group was conducted in an open format with rolling admission, allowing participants to join at any session. The intervention consisted of five sessions, each lasting approximately 90 min. The sessions were led by a licensed psychologist (first author AK) and followed a structured manual. Core components of the intervention included:

- Psychoeducation about trauma and stress reactions
- Normalization of common posttraumatic responses
- Emotion regulation strategies
- Coping and self-care strategies
- The role of social support and communication with relatives

Participants were encouraged to attend all five sessions. See Table 1 for the structure and content of the Coping School.

Table 1
The structure and content of the Coping School.

Module	Theme	Content
1	Adverse events and the brain	Intrusive symptoms and fear structures: flashbacks, nightmares, and intrusive memories How to support oneself and a close person in managing intrusive symptoms Natural recovery When the brain shuts down – dissociation Managing stress-related reactions
2	Adverse events and the body	Common physical symptoms following a traumatic event Helpful routines for taking care of the body Sleep after a traumatic event Advice for improving sleep quality Strategies for nightmares or awakenings accompanied by panic sensations
3	Breathing and stress management	Stress following a traumatic event Everyday coping strategies Recovery and restoration Exercises to reduce stress and promote recovery Panic attacks and intense anxiety Grounding exercise
4	Adverse events and avoidance	What constitutes avoidance Avoidance following a traumatic event How avoidance affects well-being Safety behaviors Strategies for reducing external avoidance Internal avoidance Strategies for reducing internal avoidance
5	Relationships and emotions	Emotions that persist after traumatic events Components of emotions and their impact on relationships How to manage difficult emotions How significant others can support someone experiencing difficult emotions

2.5. Measures

Three self-report scales were used to assess trauma-related symptoms as well as anxiety and depression, as indicators of potential adverse effects, at four time points: pre intervention (baseline), mid intervention, immediately after the intervention, and three months post-intervention.

1. The Posttraumatic Stress Disorder Checklist for DSM-5 (PCL-5: Blevins et al., 2015): This self-report instrument is designed to assess the presence and severity of posttraumatic stress symptoms according to DSM-5 criteria. Participants were instructed to focus on the target trauma identified during the assessment phase when completing the questionnaire. The PCL-5 consists of 20 items covering the four core symptom clusters of PTSD: intrusions, avoidance, negative alterations in cognition and mood, and hyperarousal. Each item is rated on a 5-point scale (0 = "not at all" to 4 = "extremely"), with higher total scores indicating greater symptom severity (total score range 0–80). The PCL-5 has demonstrated strong reliability and validity and is widely used both clinically and in research to monitor changes in trauma-related symptoms over time.

2. The Impact of Event Scale-Revised (IES-R: Chang et al., 2024): The IES-R is a self-report measure of subjective stress related to a specific traumatic event. The scale contains 22 items across three subscales: intrusion, avoidance, and hyperarousal. Participants rate the extent to which they have been affected by each symptom over the past seven days on a 5-point scale (0 = "not at all" to 4 = "extremely"). The IES-R is frequently used to complement the PCL-5, providing a more nuanced picture of how trauma affects daily functioning and subjective distress, and is particularly useful for tracking short-term changes.

3. The Hospital Anxiety and Depression Scale (HADS: Zigmond & Snaith, 1983): The HADS is a self-report questionnaire designed to assess symptoms of anxiety and depression in non-psychiatric populations. It consists of 14 items, with seven items measuring anxiety (HADS-A) and seven items measuring depression (HADS-D). Each item is rated on a 4-point scale (0 = "not at all" to 3 = "very often"), and higher scores indicate greater symptom severity. HADS is widely used in both clinical and research contexts to monitor changes in general psychological distress and to capture potential adverse effects of interventions.

2.6. Procedure

Recruitment took place at primary care units in Region Värmland, Sweden. Clinicians identified potential participants during routine clinical contacts and informed them about the Coping School. Interested

patients were referred to a psychologist, who conducted an assessment interview to determine eligibility. Eligible patients and their relatives received written and oral information about the study and provided informed consent prior to participation. Following completion of the intervention, participants were invited to take part in individual semi-structured interviews focusing on their experiences of the intervention, including perceived benefits, limitations, and suggestions for improvement. Clinicians who had been involved in identifying and referring patients were also invited to participate in individual semi-structured interviews regarding their views of the intervention, the recruitment process, and perceived barriers and facilitators to implementation (see Appendix for the interview guides).

2.7. Data analysis

Given the small sample size, quantitative data were analyzed descriptively. Individual symptom trajectories over time were examined and compared to baseline levels. No inferential statistical analyses were conducted. Qualitative data from participant and clinician interviews were analyzed using thematic analysis in line with Braun and Clarke (2021). The analysis followed six phases: familiarization with the data, generation of initial codes, searching for themes, reviewing themes, defining and naming themes, and producing the report. An inductive, semantic approach was used, and themes were identified based on patterns of meaning across the data set.

3. Results

3.1. Patients' perspectives

Of the four patients who initially consented to participate, three completed the intervention and the three-month follow-up assessments. Individual symptom trajectories varied across participants. At follow-up, one participant showed a clear reduction in trauma-related symptoms compared to baseline. One participant showed largely unchanged symptom levels over time, while one participant reported a slight increase in symptoms at follow-up. Given the very few participants, the results are presented descriptively in Table 2.

The thematic analysis of participants' interviews resulted in two overarching themes: *Breaking the pattern of avoidance* and *Implementing new skills.*

Table 2
Participants' individual and average scores across timepoints.

Outcome measures	Pre-Intervention	Mid-intervention	Post-intervention	Three-month follow-up
PCL-5				
A	36.00	41.00	35.00	33.00
B	42.00	31.00	38.00	18.00
C	49.00	58.00	48.00	53.00
M	42.33	43.33	40.33	34.67
IES-R				
A	47.00	53.00	46.00	42.00
B	46.00	36.00	43.00	29.00
C	56.00	60.00	59.00	61.00
M	49.67	49.67	49.33	44.00
HADS-anxiety				
A	11.00	13.00	12.00	11.00
B	15.00	10.00	8.00	7.00
C	16.00	18.00	16.00	16.00
M	14.00	13.67	12.00	11.33
HADS-depression				
A	3.00	5.00	2.00	2.00
B	13.00	10.00	8.00	6.00
C	15.00	15.00	12.00	13.00
M	10.33	10.00	7.33	7.00

Note. PCL-5 = The Posttraumatic Stress Disorder Checklist for DSM-5; IES-R = The Impact of Event Scale-Revised; HADS = The Hospital Anxiety and Depression Scale.

3.1.1. Breaking the pattern of avoidance

Participants described the psychoeducational content as helpful in understanding their reactions to adverse events. Learning about common trauma responses contributed to a sense of normalization and reduced self-blame. Participants reported that the intervention provided a language for describing their experiences and facilitated conversations with relatives. The psychoeducation focused on common reactions to trauma and general underlying mechanisms (e.g., avoidance) and, despite its general nature, enabled participants to draw conclusions about themselves, even though the individual's specific symptoms or unique traumatic experiences were not the focus.

> […] the fourth time, when we had the module on avoidance … that's what I noticed a lot, because I hadn't thought about it before. As a reaction because of what happened. And maybe I need to look at it a bit more. To see how I should break that part. (#1)

Breaking patterns of avoidance – such as avoiding emotions and memories – is the core of many psychological treatment approaches. Becoming aware of how one's active involvement in maintaining psychological suffering can be a crucial eye-opener. It increases confidence in one's ability to influence well-being as it makes it possible to engage in different internal and external behaviors, which in turn affect depressive and anxiety symptoms.

> Yes, well, that's what I'm saying, I hadn't previously thought about linking things that I've avoided… Like traveling to my home country, that – I really want to go, but at the same time I don't. So, there are mixed feelings, and I avoid going there. (#2)

Making patterns of avoidance visible does not automatically lead to new behaviors. However, it might make it clearer to patients why they behave the way they do, which can be an important first step toward change.

3.1.2. Implementing new skills

While participants appreciated the content of the intervention, they also described the five-session format as overly condensed. Some participants felt that important therapeutic elements – such as sufficient time for reflection, repetition, and discussion – were lost due to time constraints. Participants expressed a desire for either more or longer sessions to allow for deeper processing and skills practice.

Participants described having succeeded in implementing new and helpful skills based on the information and tips they received. Participants were encouraged to give themselves time to process the content, both between different sections during the group sessions and between sessions. This was intended to encourage patients to independently reflect on the content and its relevance to them personally.

> So, I felt a bit like, I said that it was a bit difficult to implement it. And one thing I mentioned when we had the school was that I would try to calm myself down in the evening before going to sleep. I managed, from the time we had that lecture, to do it two or three times. So, I think that it, it is good, but that you need to focus a bit more on what suits a person to introduce into their life. (#1)

Although there was an overarching theme of successfully implementing skills independently in everyday life, there were also descriptions of difficulties related to this process. One of the aims of Coping School was to provide participants with clear information and tips on concrete strategies in a compact format that patients could also return to in the future. A potential pitfall of this approach is that it may be difficult to find the time to try out or fully understand everything, which may evoke feelings of inadequacy and, in turn, reduce participants' readiness for change.

As I said, it has been very good – I think I'm repeating myself – but it really has been very good. I think so. It has been a bit dense, but as I said, you've had time to process it. So… No, but. And then, of course, people are different, but maybe a bit more… discussions, I mean, so that you don't have to share what you've been through but still share a bit more. So maybe you feel a bit more, a bit less alone in what you're going through. (#3)

3.2. Clinicians' perspectives

Eleven clinicians participated in interviews regarding their perspectives on the Coping School and the recruitment process. The thematic analysis identified three main themes: *A limited population of eligible patients, Hesitation toward group treatment*, and *Organizational barriers to collaboration.*

3.2.1. A limited population of eligible patients

In the interviews, many clinicians expressed surprise at the difficulty of recruiting suitable participants. Clinicians described the target group as relatively small within primary care. Many patients with trauma-related symptoms were either too severely affected and therefore referred to specialized psychiatric care, or experienced symptoms that were considered too mild to warrant referral to a group intervention. All interviewed clinicians stated that they were positive toward the intervention and perceived its content as appropriate and well aligned with the primary care mandate as formulated in governing policy documents. All interviewees emphasized the need for collaboration between primary care units and expressed the view that such collaboration should be feasible in practice. There was also a consistent perception that the need for interventions targeting relatives is substantial. At the same time, clinicians highlighted that the group of patients suitable for the Coping School is small.

I actually haven't had a patient that I've felt was appropriate, so unfortunately, I haven't discussed it with patients. That has a lot to do with the exclusion criteria and the fact that many of the patients we see have experienced traumatic events, but that isn't their main problem now – so a group like this doesn't really fit right now. And then there are those patients who very clearly meet criteria for PTSD and who absolutely would not be suitable for a group format. So, there are really two sides: either the traumatic experiences aren't the focus, or the clinical picture makes group treatment unsuitable – or they have a very strong need to process the event in depth. They want to talk about the event itself, rather than focusing on coping strategies like in this intervention. So no, it doesn't really fit. (#10)

Furthermore, several of the interviewed clinicians identified the inclusion criterion requiring a Criterion A trauma as a barrier to recruitment.

I'm thinking that maybe it's precisely this Criterion A. (laughs) If there had been a broader entry point to the project, I think there would have been more patients included – more opportunities to capture people. (#2)

Several clinicians' responses suggest that they perceived Coping School as potentially suitable even for patients who had not experienced events meeting the formal criteria for trauma. Many depressive and anxiety disorders can often be traced by patients to events, situations, or relationships that were difficult to manage or process – for example, growing up with a parent with mental health problems and later developing an anxiety disorder – where the symptom presentation is not best captured by the criteria for PTSD.

Of the eleven individuals referred to Coping School, seven were assessed as not having a symptom profile suitable for the intervention and/or as not having experienced a Criterion A trauma. This suggests that clinicians' initial perceptions of patients' needs may differ from those identified through more in-depth assessment, given that trauma-focused treatment presupposes the presence of a qualifying traumatic event.

I also think there might be, sort of, some uncertainty about one's own assessment, at least partly. I mean, from our side, when we're the ones making referrals – assessing whether patients are suitable for the program or whether they meet the criteria. I think we see a lot of borderline cases, people who don't quite meet all the criteria. (#1)

3.2.2. Hesitation toward group treatment

Clinicians expressed reluctance to motivate patients to participate in group-based interventions, particularly when patients were perceived as vulnerable. Concerns included fear of causing harm, uncertainty about patients' readiness for group participation, and assumptions about patients' preferences for individual treatment. The picture of the patient group is heterogeneous. Some clinicians perceive the group as generally easy to work with, while others describe it as both difficult to assess and to treat. Several clinicians reported having experienced negative patient reactions to the group format, and some explicitly stated that they believe patients should have the right to choose – and to opt out of – treatment interventions.

Yes, so that's one thing I can suggest – written exposure therapy. And not everyone wants to engage in that kind of treatment, so you can start by working with what they're dealing with right now, whether it's anxiety or something else. And then, if you notice that there isn't sufficient improvement, you can bring it up again and see whether they might want to try treatment then. But it really has to be voluntary. (#7)

Overall, perceptions of group treatment were positive, with high confidence that the format itself is helpful for participating patients and that this patient group would benefit from such an intervention. At the same time, there was a widespread perception that patients in general – and perhaps patients who have experienced trauma in particular – are difficult to recruit. Here, clinicians' assessments, in which they generally view group treatment as helpful, clash with the principle that patients have the right to decline and choose alternative interventions.

The people I talked to were hesitant, most of them, and that was because it was a group format. There was resistance to talking in front of others about what they had been through… many patients still connected it to that in some way. That they would end up there and that they would somehow have to talk about how they feel. (#2)

Some clinicians stated that they could have done more to motivate potential participants. This highlights a boundary issue concerning responsibility and ethical practice. A pattern emerges in which clinicians and organizations assume responsibility for accommodating patient preferences, even when doing so is not necessarily aligned with the primary care mandate or with the principle of cost-effectiveness that underpins the Swedish health care system.

Absolutely, and I think it's often the case that patients don't want to go into that. And then maybe they don't really improve and don't get the outcome you're hoping for, and then you go back again and say, well, maybe you do need to look at this after all? And they say no I don't know. I think they're just not very willing. So, you're not really sure – should we have pushed it more? We can see that you're still experiencing quite a bit of anxiety. I think it's a bit tricky – or difficult, I guess. (#3)

3.2.3. Organizational barriers to collaboration

Clinicians described a lack of practical conditions for collaboration and service development, including high workload, time constraints, and limited opportunities for joint planning across professional roles and units. These factors were perceived as hindering both recruitment and

the implementation of new interventions. They were largely in agreement that collaboration between counseling units would, in general, be positive and feasible, and that Coping School could constitute a cost- and resource-efficient intervention.

> I mean, I really think collaboration is necessary, because it's not cost-effective to offer the same type of treatment at every primary care center, and it's difficult to find enough patients for these treatments. So, it's good if one unit offers Coping School and another offers something else, and that we support each other – that there's somewhere to refer these patients. Right now, it feels like management expects us to treat everything and offer all kinds of group treatments. But first, there may not be enough participants at a single primary care center for group treatments. And second, no one has the capacity to run a large number of different treatments at that level. So yes, I think collaboration would be very, very good. (#6)

When clinicians described what they believed may have hindered recruitment to Coping School, they pointed to a lack of everyday contact between counseling units, which makes it difficult both to consult colleagues and to remember the intervention. A further complicating factor was that clinicians are geographically dispersed, a challenge that becomes even more pronounced when geographically separated clinicians do not know one another and often belong to different professional backgrounds.

> Yes, it seems like the primary care centers work in slightly different ways. We're different sizes and have different structures for how we work. So of course, time would need to be set aside to sit down and talk about collaboration and what it would look like in practice. And that time maybe just isn't available right now, with budget cuts and all. We don't really have any breathing room in the system, in our schedules to make that happen, unfortunately. (#4)

Within this service area, counseling units meet digitally for one hour on two occasions per term, and once per year for a full-day meeting together with all clinicians within psychosocial services across the entire region. Thus, the organization offers limited opportunities for clinicians to get to know colleagues at other units beyond their own. Many clinicians highlighted time pressure and a strong focus on productivity as factors that influence daily work and make it difficult to remember or implement new ways of working.

> …if we had met more often, or just had that kind of dialogue, then – if we had worked at the same workplace – I would probably have gone to you right away afterward and asked, 'What do you think about this patient?' Because these assessments aren't always straightforward. Even if you read the diagnostic criteria, it's not that simple. Maybe then I would have included her after all, but instead I just thought, 'No, this doesn't fit,' and then you have to move on. (#6)

4. Discussion

Despite the small sample size, the recruited participants attended all group sessions and assessment points. This may indicate that the group format could be feasible for a broader segment of the patient population. High adherence is important for services to justify group-based interventions. It is also possible that adherence reflects that participants received appropriate interventions that generated interest and motivation to continue, which would suggest accurate clinical assessment in matching the right individual to the right intervention. Within primary care, there is a strong emphasis on accessibility, which entails both conducting assessments as promptly as possible and ensuring their accuracy. It is noteworthy that none of the participants chose to include a significant other, despite this being encouraged and an explicit aim of the intervention. This may be attributable to the small sample size, but it may also reflect how individuals suffering from mental health problems relate to their difficulties and to close relationships. Alternative formats may be needed to facilitate social support from significant others.

At the same time, the very small sample size constitutes a major limitation and requires caution in interpretation. With only four included participants, of whom three completed the intervention and follow-up, the study cannot support conclusions regarding efficacy or effectiveness. The small sample also limits internal validity, as individual variation has a large impact on observed patterns and makes it difficult to distinguish intervention-related change from random fluctuation or idiosyncratic participant characteristics. External validity is likewise highly restricted, as the findings cannot be assumed to generalize to the broader population of primary care patients exposed to adverse events. Accordingly, the present study should be interpreted strictly as a proof-of-concept feasibility investigation rather than an evaluation of clinical efficacy.

The group format was perceived as a potential barrier by clinicians, even though they also recognized multiple therapeutic and organizational advantages. Providing interventions for significant others was perceived as an area of substantial need. One theme in the analysis suggested that a limited pool of suitable patients may underlie recruitment difficulties. Of the 11 patients referred, only four were deemed suitable following assessment. Together with clinicians' uncertainty regarding assessment and treatment, this may reinforce the perception held by some clinicians that the target population is small, despite the intervention itself being broadly applicable.

The lack of participation by significant others is particularly important in relation to the intervention rationale. Coping School was developed not primarily to replace established trauma-focused treatments, but to offer a brief, accessible, and potentially scalable psychoeducational intervention in primary care for individuals with stress-related difficulties after adverse events, including those who may not yet need, be ready for, or have access to more intensive trauma-focused treatment. A further intended advantage was the possibility of involving significant others in order to strengthen social support and shared understanding. However, since no participants chose to include a significant other, this proposed added value was not realized in the present study. The discrepancy between the intended relational component and the actual implementation suggests that this aspect of the intervention may need to be reformulated, offered in a different format, or evaluated separately in future studies.

An evidence-based approach and critical thinking are essential for clinicians seeking to deliver effective interventions under local conditions. The expectation that primary care can treat mild-to-moderate mental health problems across a wide range of conditions within a limited number of sessions may be more utopian than realistic. Psychiatric conditions exist along a dimensional continuum rather than discrete categories, and treatment needs likely do as well. The gap between primary care (typically 8–12 sessions) and specialist psychiatry (often 25+ sessions) suggests unmet needs in the intermediate range. Centralizing counseling services could potentially allow for greater specialization and resilience through clinician heterogeneity. The findings mirror previous research on implementation barriers in primary care, involving factors at the clinician, patient, organizational, and political levels. The study highlights the complex situation facing primary care, expected to be both person-centered and efficient while lacking opportunities for collaboration and development. These findings underscore the importance of thoroughly analyzing challenges and barriers prior to implementation.

Group-based psychological treatment is currently used in both primary care and psychiatric settings. Group formats may offer several advantages for patients, such as normalization of their difficulties, increased sense of belonging, and engagement with others, which may enhance motivation and commitment to treatment (Bengtsell & Thoursie, 2023). If effective, open group formats may provide substantial benefits for both patients and health care services by further increasing accessibility. Silberleitner et al. (2021) describe the potential of brief

group formats in primary care for psychoeducation and skills training – such as emotion regulation – delivered over three sessions, with the option for patients to attend additional cycles if further practice is needed. Tourigny and Hébert (2007) compared open and closed group formats for adolescent girls who had been exposed to sexual abuse and found no significant differences between the two formats. Nevertheless, further research is needed to determine whether open groups are as effective as closed groups, given the many potential advantages associated with open formats.

The potential value of Coping School should be understood in relation to service gaps rather than superiority over existing evidence-based trauma treatments. In a stepped-care context, a brief psychoeducational group may serve as a low-threshold intervention for patients with subthreshold or mixed stress-related symptoms, as an early supportive intervention while further assessment is ongoing, or as an interim intervention during waiting times for more specialized care. Its possible comparative advantage lies in accessibility, brevity, and scalability within routine primary care rather than in trauma processing as such. This also means that the intervention is unlikely to be appropriate for all trauma-exposed patients, particularly those with more severe symptoms, high instability, marked dissociation, acute suicidality, severe comorbidity, or a clear need for individualized trauma-focused treatment.

Previous research highlights that recruiting patients to psychological interventions in primary care is influenced by multiple, often interacting, barriers. For example, Mason et al. (2007) found that primary care physicians may experience hesitancy to recruit patients due to perceived vulnerability and a desire to protect them, alongside feelings of limited competence in discussing research participation and challenges integrating recruitment into routine clinical practice. Similarly, Titzler et al. (2020) identified barriers and facilitators to referral for blended psychotherapy at multiple levels, encompassing the individual clinician, patient characteristics, organizational context, and broader socio-political factors. These findings underscore that recruitment challenges are rarely attributable to a single factor but reflect a complex interplay between personal, organizational, and systemic influences.

The study design also limits interpretation of symptom change. Because the study used a within-group design without a control condition, it is not possible to determine whether observed changes were attributable to the intervention itself, spontaneous improvement, regression to the mean, repeated measurement, or other contextual factors. This is particularly relevant given the heterogeneous symptom trajectories across participants. Any apparent improvement should therefore be interpreted cautiously and not as evidence of treatment effect.

In addition, potential contraindications and clinical limitations should be acknowledged more explicitly. Group-based psychoeducation may provide insufficient containment for some trauma-exposed individuals, especially if symptoms are severe or if participation in a group evokes distress without offering sufficient opportunity for individualized processing. The format may also represent a mismatch for patients whose primary need is trauma-focused exposure, cognitive processing, or more comprehensive psychiatric treatment. Careful assessment is therefore essential in order to identify which patients may benefit from a brief psychoeducational intervention and which should instead be referred to more intensive or specialized care.

The potential efficiency and effectiveness of the Coping School could be evaluated in future larger trials, for example by expanding recruitment to include patients from a wider area. The materials could also function as clinician support in individual treatment contexts. Other formats could be explored, such as collaborations between regional and municipal services targeting young adults, who often maintain close contact with significant others. Given long waiting times for trauma-focused treatment in outpatient psychiatry, Coping School could also serve as an interim intervention while patients are waiting for treatment. Given the lack of participation by significant others, future research should explore how interventions for relatives can be designed. For example, brief psychoeducational interventions for relatives alone may lower participation thresholds. It may also be valuable to identify patients encountered by other healthcare professionals who present with physical or psychological symptoms potentially linked to adverse life events.

Future studies should include substantially larger samples and, ideally, a comparison condition in order to evaluate feasibility, safety, and preliminary effectiveness more rigorously. Such studies would also make it possible to examine which subgroups are most likely to benefit, under what clinical circumstances the intervention is appropriate, and whether the significant-other component contributes meaningful added value.

4.1. Conclusions

The study highlights how variations in clinician competence affect assessment and treatment. Clinicians expressed positive attitudes toward collaboration but identified substantial barriers. A tentative conclusion is that primary care within the studied service area faces significant challenges and requires improved working methods. Achieving this will likely require better practical conditions for discussing needs and anchoring new strategies among clinicians engaged in direct patient care. The study supports previous findings regarding recruitment barriers in primary care psychological treatment research. Greater coordination among primary care counseling services could enable both breadth and depth of care, allowing clinicians to specialize rather than each being required to address all conditions. To meet public and policy expectations, clinicians must be provided with adequate resources for service development. Public investment is therefore warranted but must be applied judiciously and grounded in scientific evidence. However, conclusions regarding clinical effectiveness must be avoided given the very small sample and uncontrolled design. The present findings primarily inform the feasibility challenges of implementing and recruiting to this type of intervention in primary care. Larger and methodologically stronger studies are needed before any conclusions can be drawn about patient benefit, comparative value, or broader applicability.

Author contributions

Siri Jakobsson Støre was the principal investigator. Siri Jakobsson Støre and Amanda Karlsson conceptualized the study. Maria Unenge Hallerbäck and Charlotte Bäccman contributed to the initial planning and the ethics application. Amanda Karlsson was responsible for data collection and data analysis, under the supervision of Siri Jakobsson Støre. Siri Jakobsson Støre and Amanda Karlsson wrote the first draft of the manuscript. All authors revised and approved the final manuscript.

Data availability

Data are available from the first author upon reasonable request.

Funding

This research received no external funding.

Declaration of competing interest

The authors declare that they have no competing interests.

Supplementary materials

Supplementary material associated with this article can be found, in the online version, at doi:10.1016/j.ejtd.2026.100685.

References

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders: Dsm-5-tr* (5th edition). American Psychiatric Association Publishing. text revision.

Are, F., Espeleta, H., Newman, C., Ridings, L. E., Wallis, E., & Hanson, R. F. (2021). Evidence-based interventions for posttraumatic disorder in primary healthcare settings. *Handbook of evidence-based prevention of behavioral disorders in integrated care: A stepped care approach* (pp. 171–192). Springer. s.

Bengtsell, L., & Thoursie, A. (2023). *Leda behandling i grupp*. Studentlitteratur.

Bleiberg, K. L., & Markowitz, J. C. (2019). Interpersonal psychotherapy for PTSD: Treating trauma without exposure. *Journal of Psychotherapy Integration, 29*(1), 15–22.

Blevins, C. A., Weathers, F. W., Davis, M. T., Witte, T. K., & Domino, J. L. (2015). The posttraumatic stress disorder checklist for DSM-5 (PCL-5): Development and initial psychometric evaluation. *Journal of Traumatic Stress, 28*(6), 489–498.

Bowen, D. J., Kreuter, M., Spring, B., Cofta-Woerpel, L., Linnan, L., Weiner, D., Bakken, S., Kaplan, C. P., Squiers, L., Fabrizio, C., & Fernandez, M. (2009). How we design feasibility studies. *American Journal of Preventive Medicine, 36*(5), 452–457.

Braun, V., & Clarke, V. (2021). *Thematic analysis: A practical guide*. Sage Publications.

Center for Mental Health Competence. (2023). En stegvis vårdmodell [Swedish] Accessed from: https://www.kompetenscentrumpsykiskohalsa.se/material-vuxna2/vuxna/.

Chang, S., Kim, W.-H., Jung, Y.-E., Roh, D., Kim, D., Chae, J.-H., & Park, J. E. (2024). Clinical utility of impact of event scale-revised for diagnostic and statistical manual of mental disorders – fifth edition posttraumatic stress disorder. *Psychiatry Investigation, 21*(8), 870–876.

Frans, Ö., Rimmö, P. A., Åberg, L., & Fredrikson, M. (2005). Trauma exposure and post-traumatic stress disorder in the general population. *Acta Psychiatrica Scandinavica, 111*(4), 291. -290.

Fredette, C., El-Baalbaki, G., Palardy, V., Rizkallah, E., & Guay, S. (2016). Social support and cognitive–behavioral therapy for posttraumatic stress disorder: A systematic review. *Traumatology, 22*(2), 131–144.

Hafstad, G. S., Thoresen, S., Wentzel-Larsen, T., Maercker, A., & Dyb, G. (2017). PTSD or not PTSD? Comparing the proposed ICD-11 and the DSM-5 PTSD criteria among young survivors of the 2011 Norway attacks and their parents. *Psychological Medicine, 47*(7), 1283–1291.

Hood, C. O., Southward, M. W., Bugher, C., & Sauer-Zavala, S. (2021). A preliminary evaluation of the unified protocol among trauma-exposed adults with and without PTSD. *International Journal of Environmental Research and Public Health, 18*(21), Article 11729.

Hyland, P., Shevlin, M., McNally, S., Murphy, J., Hansen, M., & Elklit, A. (2016). Exploring differences between the ICD-11 and DSM-5 models of PTSD: Does it matter which model is used? *Journal of Anxiety Disorders, 37*, 48–53.

Kartha, A., Brower, V., Saitz, R., Samet, J. H., Keane, T. M., & Liebschutz, J. (2008). The impact of trauma exposure and post-traumatic stress disorder on healthcare utilization among primary care patients. *Medical Care, 46*(4), 388.

Lewis, C., Roberts, N. P., Andrew, M., Starling, E., & Bisson, J. I. (2020). Psychological therapies for post-traumatic stress disorder in adults: Systematic review and meta-analysis. *European Journal of Psychotraumatology, 11*(1), Article 1729633.

Marx, B. P., Hall-Clark, B., Friedman, M. J., Holtzheimer, P., & Schnurr, P. P. (2023). The PTSD criterion A debate: A brief history, current status, and recommendations for moving forward. *Journal of Traumatic Stress, 37*(1), 5–15.

Mason, V., Shaw, A., Wiles, N., Mulligan, J., Peters, T., Sharp, D., & Lewis, G. (2007). GPs' experiences of primary care mental health research: A qualitative study of the barriers to recruitment. *Family Practice, 24*(5), 518–525.

Michel, P.-O., Lundin, T., Bergh Johannesson, K., Nilsson, D., & Arnberg, F. (2018). *Psykotraumatologi*. Studentlitteratur.

National Board of Health and Welfare. (2021). Nationella riktlinjer för vård vid depression och ångestsyndrom - stöd för styrning och ledning [Swedish] Accessed from: https://www.socialstyrelsen.se/kunskapsstod-och-regler/regler-och-riktlinjer/nationella-riktlinjer/riktlinjer-och-utvarderingar/depression-och-angest/.

National Center for PTSD & National Child Traumatic Stress Network. (2020). *Skills for psychological recovery - Field Operation guide*. The National Child Traumatic Stress Network Accessed from https://www.nctsn.org/resources/skills-for-psychological-recovery.

O'Donnell, M. L., Lau, W., Chisholm, K., Agathos, J., Little, J., Terhaag, S., Brand, R., Putica, A., Holmes, A. C. N., Katona, L., Felmingham, K. L., Murray, K., Hosseiny, F., & Gallagher, M. W. (2021). A pilot study of the efficacy of the unified protocol for transdiagnostic treatment of emotional disorders in treating posttraumatic psychopathology: A randomized controlled trial. *Journal of Traumatic Stress, 34*(3), 563–574.

Schnyder, U., Ehlers, A., Elbert, T., Foa, E. B., Gersons, B. P., Resick, P. A., Shapiro, F., & Cloitre, M. (2015). Psychotherapies for PTSD: What do they have in common? *European Journal of Psychotraumatology, 6*(1), Article 28186.

Silberleitner, N., Farnsworth von Cederwald, A., & Robinson, P. J. (2021). *Integrerad primärvård: Principer, färdigheter och rutiner för hela vårdcentralens arbete med beteenderelaterad ohälsa*. Natur & Kultur.

Southwick, S. M., Charney, D. S., & DePierro, J. M. (2023). *Resilience: The science of mastering life's greatest challenges*. Cambridge University Press.

Stein, D. J., McLaughlin, K. A., Koenen, K. C., Atwoli, L., Friedman, M. J., Hill, E. D., Maercker, A., Petukhova, M., Shahly, V., van Ommeren, M., Alonso, J., Borges, G., de Girolamo, G., de Jonge, P., Demyttenaere, K., Florescu, S., Karam, E. G., Kawakami, N., Matschinger, H., Okoliyski, M., Posada-Villa, J., Scott, K. M., Viana, M. C., & Kessler, R. C. (2014). DSM-5 and ICD-11 definitions of posttraumatic stress disorder: Investigating "narrow" and "broad" approaches. *Depression and Anxiety, 31*(6), 494–505.

Swedish Psychological Association. (2020). Principer för Evidensbaserad Psykologisk Praktik (PBPP) [Swedish] Accessed from: https://www.psykologforbundet.se/rad-och-stod/din-yrkesutovning/stod-i-din-yrkesvardag/principer-for-ebpp/.

Titzler, I., Berking, M., Schlicker, S., Riper, H., & Ebert, D. D. (2020). Barriers and facilitators for referrals of primary care patients to blended internet-based psychotherapy for depression: Mixed methods study of general practitioners' views. *JMIR Mental Health, 7*(8), Article e18642.

Tourigny, M., & Hébert, M. (2007). Comparison of open versus closed group interventions for sexually abused adolescent girls. *Violence and Victims, 22*(3), 334.

Ungar, M. (2013). Resilience, trauma, context, and culture. *Trauma, Violence & Abuse, 14*(3), 255–266.

Vogt, D. S., King, D. W., & King, L. A. (2007). Risk pathways for PTSD. *Handbook of PTSD: Science and practice*.

World health Organization. (2025). International Statistical Classification of Diseases and Related Health Problems (ICD) Accessed from: https://www.who.int/standards/classifications/classification-of-diseases.

Zigmond, A. S., & Snaith, R. P. (1983). The hospital anxiety and depression scale. *Acta Psychiatrica Scandinavica, 67*(6), 361–370.